3.15 Tracheostomy Care

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • Points at his throat to show it is uncomfortable **Objective** • Thick secretions in the trach tube • Noisy breathing	**ACTUAL** **Ineffective airway clearance** related to increased secretions and artificial airway as evidenced by noisy breathing and visible secretions.	Tracheostomy stays patent and the airway clear.	• Suction when needed, using sterile technique. • Clean or change the inner cannula as policy says. • Humidify the air. • Keep a spare tube, the obturator and a tracheal dilator at the bedside.	Airway clear: **goal met**.
Subjective • "I do not want people to see my neck." **Objective** • Will not look at the stoma • Hides his neck with his hand when visitors come	**ACTUAL** **Disturbed body image** related to tracheostomy as evidenced by avoidance of looking at stoma.	Looks at the stoma and starts to help care for it.	• Let him talk about how he feels. • Bring him into his care little by little. • Suggest a scarf or stoma cover for when he goes out.	Helps with stoma care: **goal met**.
Subjective • *No specific complaint* **Objective** • Surgical stoma • Secretions around the site	**RISK** **Risk for infection** as evidenced by risk factors: open stoma and secretions.	Stoma stays clean, with no signs of infection.	• Clean the stoma and change the dressing as policy says. • Change the ties when soiled, leaving room for one finger. • Look for redness or pus.	Stoma clean: **goal met**.